Institutional Reform and Original-Drug Innovation: Organizational Routes, Commercialization Assignment, and the MAH System in China

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Abstract

Master skeleton only. This file fixes the article architecture, section interfaces, terminology discipline, and appendix mapping so that subsequent section-level revisions can be inserted without changing the paper's current object. The central mechanism remains the decline in τ_{ext}, the route-specific governance and compliance friction of external commercialization, rather than a generic policy wedge. Empirical claims, calibrated parameters, and quantitative results are intentionally left to later verified drafts.

Keywords: MAH reform; pharmaceutical innovation; commercialization assignment; organizational routes; heterogeneous firms; contract manufacturing.
JEL codes: D22; L23; L65; O31; O38.

1 Introduction

Role in the paper. This section introduces the paper's object, motivates the pharmaceutical setting, states the central mechanism, positions the contribution relative to the relevant literatures, and previews the structure of the paper. It should define the problem at the level of organizational routes from invention to commercialization, not at the level of a generic policy shock.

Locked-in object. The introduction must remain centered on the claim that the MAH reform changes the organizational route through which a research-originated original-drug idea is commercialized, especially by making external commercialization feasible under retained authorization status.

What this section should do in later drafts.

- Motivate why commercialization, rather than invention alone, is the relevant bottleneck.
- State why MAH should not be modeled as a direct scientific-productivity shock or a generic demand shock.
- Preview the three organizational forms A, B, and C at a high level only.
- State the paper's mechanism, modeling, and empirical contributions without overloading the introduction with institutional detail.

Interface to Section 2. The final paragraph of the introduction should hand off to the institutional section by posing the narrow question: what exactly does MAH change in economic terms, and what can the data identify?

2 Institutional Setting and Identification Boundary

The economic question of this paper is not whether regulatory reform improves innovation in a broad and undifferentiated sense. It is narrower and more disciplined. We study whether the Marketing Authorization Holder (MAH) reform changes the organizational route through which an original-drug idea moves from invention to commercialization, and whether that change increases the set of research-originated projects that can be realized as marketable products.

This distinction matters for both interpretation and modeling. In the present setting, the key constraint is not simply the scientific difficulty of discovering a promising idea. The deeper bottleneck is whether a research-originated idea can be brought to market without forcing the innovator to internalize the entire downstream production chain. In other words, the central margin is organizational implementation rather than scientific discovery alone. The reform is therefore most naturally understood as changing the conditions under which commercialization can be carried out across firm boundaries, not as a generic increase in demand, a direct rise in scientific productivity, or a reduced-form shift in all firm payoffs at once.

2.1 Institutional environment and the substantive distinction between generic and original drugs

We study China's pharmaceutical industry in an environment with two medicine categories,

$$k \in \{G, O\},$$

where G denotes generic drugs and O denotes original drugs. This distinction is substantive rather than cosmetic. The paper's policy question concerns the realization of new original-drug products rather than total pharmaceutical output. Generic production remains economically important because it provides cash flow, manufacturing demand, and a source of revenue for integrated and manufacturing-specialized firms. But the reform studied here acts primarily on the commercialization environment for original-drug ideas.

This focus follows from the industry's broader upgrading problem. As the industry moves from a generic-oriented stage toward one in which original-drug development becomes more important, the ability to generate an idea and the ability to commercialize that idea need not be located inside the same organization. This separation is especially relevant when research-oriented firms can generate valuable projects but do not operate full internal manufacturing capacity. Under those conditions, the critical question becomes whether the institutional environment allows research-originated projects to be completed through a legally recognized and operationally viable external route.

This is why the paper focuses on the organization of commercialization rather than on a standard production-side channel. If the reform mainly changed factory productivity, static input demand, or product demand, then a different model would be appropriate. The mechanism emphasized here is different. The reform matters because it changes whether a research-originated project can be implemented while the innovator retains the relevant authorization position and coordinates with a qualified outside manufacturer.

2.2 What MAH changes institutionally

Let

$$M \in \{0, 1\}$$

denote the regulatory regime, where $M = 0$ is the pre-MAH regime and $M = 1$ is the post-MAH regime. The institutional content of MAH can be stated more concretely than a generic reform dummy. The reform changes who may legally hold market authorization, how production may be organized once authorization is obtained, and how responsibility is allocated when production is entrusted to another firm.

A first key step is that the reform decouples, in legal form, the authorization-holding role from the requirement of in-house production. The 2016 pilot plan already moved in that direction by allowing drug R&D institutions and scientific personnel in the pilot regions to apply for drug marketing authorization and become holders even when they were not conventional integrated drug manufacturers. For research-originating entities without production facilities, this was a basic institutional break from the older environment, in which research,

approval, and production were much more tightly bundled in practice.[1]

A second key step is that the reform does not merely allow external production in a loose sense; it legalizes a specific entrusted-production structure. Under the 2019 revised Drug Administration Law, the marketing authorization holder may either manufacture the drug itself or entrust a qualified drug manufacturer to produce it. In the entrusted-production case, the holder and the entrusted producer must sign both an entrustment agreement and a quality agreement and must strictly perform the obligations stated in those agreements. Institutionally, this means that external commercialization is not modeled as an anonymous spot-market purchase of factory services. It is a legally structured relationship with specified contractual and quality-management content.[2]

A third key step is that the reform preserves and sharpens holder responsibility rather than shifting it away from the holder. The 2019 law makes the holder the legally central entity for the drug across its lifecycle: non-clinical research, clinical trials, production and distribution, post-marketing study, and pharmacovigilance. The same law also states that the holder's legal representative and principal person are fully responsible for drug quality, and that the holder must establish a quality assurance system, assign dedicated quality-management personnel, and regularly audit entrusted manufacturers and distributors. The 2022 NMPA supervisory provisions deepen that architecture by emphasizing a full-process quality-management system and clearer responsibility allocation across key posts, while the 2023 NMPA notice on MAH entrusted production strengthens supervision of the entrusted route itself.[3]

These legal details matter for economic interpretation. Before MAH, a research-originating firm without internal manufacturing capacity faced a severe obstacle if it wanted to commercialize an original-drug project while retaining effective control and the authorization

[1]Official sources are explicit on this point. The 2016 *Pilot Plan for the Marketing Authorization Holder System* states that drug R&D institutions and scientific personnel in the pilot regions may apply for authorization and become holders; it also requires R&D institutions or scientific personnel that become holders to submit a drug-quality and safety responsibility commitment and, before market launch, documentation related to the guarantor arrangement specified by the pilot plan. See the General Office of the State Council's pilot-plan release of June 6, 2016, and the CFDA follow-up notice of July 7, 2016.

[2]See the 2019 revised *Drug Administration Law of the People's Republic of China*, especially the MAH chapter as published by the NMPA. The law states that the marketing authorization holder may self-manufacture or entrust a qualified drug manufacturer to produce the drug, and that entrusted production requires an entrustment agreement and a quality agreement.

[3]The official law text states that the holder is responsible for the drug across research, trials, production and distribution, post-marketing study, and adverse-reaction monitoring and handling; that the legal representative and principal person bear full responsibility for drug quality; and that the holder shall establish a quality assurance system and regularly audit entrusted parties. See the 2019 revised *Drug Administration Law*. For supervisory deepening, see the NMPA's 2022 *Provisions on Supervising the Implementation of the Drug Marketing Authorization Holder's Primary Responsibility for Drug Quality and Safety*, released on December 29, 2022 and effective March 1, 2023, and the NMPA's 2023 announcement on strengthening supervision of MAH entrusted production.

position. In practice, the innovator often had to move toward internal integration, rely on a more fragile organizational arrangement, or relinquish control over downstream implementation. After MAH, external commercialization through a qualified manufacturer becomes a legally recognized route under which the holder may retain authorization while production is carried out by another firm under formalized contractual and supervisory requirements.

That change should not be described as a generic policy relaxation. Its institutional content is more specific. The reform alters the legal and organizational conditions under which an external commercialization relationship can be executed. It clarifies who remains responsible, standardizes the contractual basis of entrusted production, makes cross-firm implementation more regulable and more supervisable, and thereby makes it more feasible for a research-based firm to bring a project to market without first becoming a fully integrated manufacturer.

For the purposes of this paper, the sharpest economic interpretation is therefore that MAH lowers the governance and compliance burden attached to a particular route: external commercialization of a research-originated original-drug project through a qualified outside manufacturer while the holder retains legally meaningful responsibility. The subsection that follows translates that institutional change into the route-specific friction τ_{ext}, while Appendix A provides the fuller institutional documentation.

2.3 The economic mechanism: from institutional change to τ_{ext}

This institutional interpretation leads to a disciplined modeling choice. The reform is not written as a demand shifter. It is not written as a direct increase in scientific productivity. It is not written as a generic regime dummy entering every firm payoff. Instead, the reform enters through a route-specific friction,

$$\tau_{ext}(M) \geq 0,$$

which denotes the governance and compliance cost of completing commercialization through an external qualified manufacturer while the innovator retains the relevant holder role. The present version therefore sharpens the policy primitive relative to earlier drafts: the paper's central MAH-sensitive object is the external-route friction itself.

This object is deliberately narrow. It includes the organizational and compliance burdens specific to that route, such as the execution and monitoring of the external arrangement, the compliance obligations attached to holder-side responsibility, and the cross-firm coordination required to turn a research-originated project into a marketable product. It does not represent physical manufacturing cost itself, the R&D cost of generating an idea, or an economy-wide fixed cost. This distinction is central to the paper's internal logic. Once the

reform is mapped into $\tau_{ext}(M)$, the model can cleanly distinguish three different objects: the market price of qualified external manufacturing, the technical or execution-side success of commercialization, and the institutional friction specific to the external route.

The immediate implication is straightforward. When

$$\tau_{ext}(1) < \tau_{ext}(0),$$

the expected private value of a research-originated project rises for firms that rely on external commercialization. This does not require scientific productivity to change. A project becomes more attractive because the route through which it can be completed becomes more feasible and less costly in organizational terms.

2.4 Identification boundary

A clear identification boundary follows from this logic. With standard firm-level and product-level outcome data, the paper can study whether MAH is associated with more original-drug realization, a higher share of original-drug outcomes, changes in entry composition, and reallocation across organizational forms. These are objects that can be linked to the reform in reduced-form or structural-reduced-form evidence.

What the data do not automatically reveal is a primitive route-specific friction or a primitive implementation probability by themselves. The model therefore interprets the commercialization block as a structural explanation of observed changes rather than as a primitive that is directly measured in isolation. This is an important discipline device. It prevents the paper from claiming more than the available evidence can support.

The empirical implication is that the main evidence should be organized around outcomes that are closest to the mechanism. At the first layer are measures of original-drug realization and the use or viability of external commercialization routes. At the second layer are entry composition, organizational reallocation, and producer-side outcomes in the qualified manufacturing segment. Taken together, these objects can support the interpretation that the reform operates by reducing an external commercialization friction. Taken separately, none of them should be presented as a direct estimate of τ_{ext} itself.

This mapping also changes how the paper describes organizational reallocation. The relevant question is not simply whether firms move mechanically across three labels. The more fundamental question is how original-drug projects are assigned to commercialization routes. An idea can be implemented inside an integrated firm, can be commercialized by a research-specialized firm that contracts with a qualified manufacturing specialist, can induce organizational switching, or can fail to reach implementation. The three organizational types remain useful because they organize these possibilities in a tractable way. But the paper's

first-order object is commercialization assignment.

Interface to Section 3. The model in the next section builds on this institutional mapping. It combines three elements. First, it retains an industry-dynamics shell with heterogeneous firms, endogenous entry and exit, and a stationary equilibrium perspective. Second, it distinguishes invention from implementation, so that idea generation does not automatically imply realized innovation. Third, it places a qualified contract-manufacturing market next to integrated implementation, which makes it possible to study how the reform changes the assignment of original-drug projects across routes. Under this interpretation, MAH raises original-drug innovation not because it makes all firms more scientifically productive, but because it changes the organizational feasibility of implementation.

3 Model

Role in the paper. This section translates the institutional mechanism into a formal dynamic industry model. It should preserve the current architecture already adopted in the main draft: three organizational forms, a one-dimensional capability state, static production blocks optimized out before the Bellman equations, commercialization assignment for type-B firms, and a stationary equilibrium shell.

3.1 Environment

Objects fixed at the skeleton stage.

- Time is discrete.
- Active organizational forms are A, B, and C.
- Firms carry a persistent capability state $z \in Z \subset \mathbb{R}_+$.
- The model includes the factor-price object w and the qualified contract-manufacturing price p_m.

3.2 Static production blocks

Purpose. This subsection records the paper's commitment to optimizing static production inputs out of the Bellman problem. Later full drafts should define the optimized operating-payoff objects $\bar{\pi}_A$ and $\bar{\pi}_C$ here.

Discipline. Labor, capital services, intermediates, and current manufacturing quantity should remain static objects unless a later verified draft explicitly makes one of them a state variable.

3.3 Integrated firms

Purpose. This subsection introduces type-A firms, their idea-generation choice x_A, and the resulting flow-payoff object $\Pi_A(z;\cdot)$. The later full version should define the per-idea payoff object for integrated implementation without duplicating institutional discussion from Section 2.

3.4 Research-specialized firms and commercialization assignment

Purpose. This is the core modeling subsection. It should introduce type-B firms, their idea-generation choice x_B, the set of feasible commercialization routes,

$$H = \{\text{externalize, integrate, transfer, abandon}\},$$

and the per-idea value object $\Psi_B(z; p_m, M)$.

Locked-in mechanism. The external route must remain the route on which MAH operates most directly:

$$H_B^{ext}(z; p_m, M) = \lambda_{ext}(z, p_m)R_B(z) - p_m - \tau_{ext}(M).$$

If later drafts enrich the model, they may expand internal or transfer routes, but they should not replace the central role of $\tau_{ext}(M)$.

3.5 Organizational switching and Bellman equations

Purpose. This subsection introduces switching, exit, continuation values, and the Bellman system. Later full drafts should place the value functions

$$V_A(z; M), \quad V_B(z; M), \quad V_C(z; M)$$

and the associated switching-cost objects $\kappa_{sj}(M)$ here.

3.6 Entry and stationary equilibrium

Purpose. This subsection defines entrant types, free-entry conditions, market-clearing objects, and the stationary equilibrium concept. It should make clear that the model uses a Hopenhayn-style stationary industry shell adapted to an organizational-boundary question.

Interface to Section 4. The final paragraph should state that, once the equilibrium objects are defined, the paper can state comparative-static predictions for commercialization assignment, research intensity, innovative entry, and producer-side outcomes.

4 Equilibrium Logic and Main Predictions

Role in the paper. This section states the paper's main theoretical predictions in a journal-readable way. It is not the place for full proofs or long derivations. Detailed arguments belong in the appendix.

4.1 External commercialization value

Purpose. This subsection will state the prediction that a decline in τ_{ext} raises the value of the external commercialization route and weakly expands the set of states in which that route is chosen.

4.2 Research intensity of type-B firms

Purpose. This subsection will state the prediction that a higher per-idea commercialization value raises the optimal idea-generation intensity of research-specialized firms under convex research cost.

4.3 Innovative entry

Purpose. This subsection will state the prediction that easier commercialization can increase research-oriented entry by raising the expected value of entering as a type-B firm.

4.4 Producer-side response and equilibrium ambiguity

Purpose. This subsection will record the equilibrium warning: greater demand for external commercialization need not imply a mechanically monotone effect on type-C profits once prices and market-clearing adjust.

Interface to Section 5. The final paragraph of this section should rank the empirical objects implied by the model, with commercialization assignment and original-drug realization as first-layer outcomes, and transition matrices and producer-side objects as supporting validation margins.

5 Empirical Mapping and Estimation Strategy

Role in the paper. This section maps model objects to observables and lays out the estimation logic. At the skeleton stage, it should remain explicit that empirical results are not yet inserted here.

5.1 Organizational types and key observables

Purpose. This subsection will map empirical firm categories into model types A, B, and C, and list the first-layer and second-layer empirical objects implied by the model.

5.2 Identification logic

Purpose. This subsection will explain how the joint movement of innovation outcomes, commercialization proxies, entry composition, and organizational transitions disciplines the interpretation of $\tau_{ext}(1) - \tau_{ext}(0)$.

5.3 Practical estimation and implementation

Purpose. This subsection will describe the intended estimation workflow, including state discretization, stationary-equilibrium solution, and either simulated method of moments or indirect inference. It should not report parameter estimates until a later verified draft is ready.

5.4 What the current draft does and does not claim

Purpose. This subsection should remain as an explicit safeguard against overclaiming. It will say what has been fixed at the level of object, mechanism, model structure, and empirical mapping, and what still requires verified data construction, estimation, and robustness work.

6 Conclusion

Role in the paper. The conclusion should not reopen the model or add new claims. It should simply return to the paper's central object: MAH changes commercialization assignment by lowering the route-specific friction of external commercialization, thereby potentially changing innovative entry, organizational reallocation, and the realization of original-drug products.

Broader takeaway. A later full draft may use the conclusion to connect the paper back to the broader question of how institutional design changes the organizational routes through which ideas become products in a latecomer industry.

A Institutional Details and Mechanism Clarification

This appendix expands the institutional discussion in Section 2 in a way that is closer to how a JDE paper would document the legal content of the reform. The objective is not merely to repeat that MAH "lowers external commercialization costs," but to show why that is the most disciplined economic translation of the institutional record. The basic point is that MAH did not simply remove regulation. Rather, it created a legally recognized route under which authorization can remain with the holder while production is performed by a qualified outside manufacturer, and it paired that route with explicit holder-side responsibility, contractual requirements, and supervisory expectations.[4]

A.1 Institutional evolution: from pilot to legal consolidation

The institutional evolution matters because it clarifies that MAH is not a loose industry slogan but a progressively formalized legal arrangement.

First stage: the 2016 pilot. The 2016 pilot plan already made two core ideas explicit. First, in the pilot provinces, drug R&D institutions and scientific personnel could act as registration applicants and, after obtaining market authorization, become marketing authorization holders. Second, the pilot plan made clear that the holder would not escape legal responsibility merely because production was entrusted to another entity. The plan

[4]The discussion in this appendix is anchored in four official layers of the Chinese regulatory record: the 2016 *Pilot Plan for the Marketing Authorization Holder System*, issued by the General Office of the State Council; the 2019 revision of the *Drug Administration Law of the People's Republic of China*; the NMPA's 2022 *Provisions on Supervising the Implementation of the Drug Marketing Authorization Holder's Primary Responsibility for Drug Quality and Safety*, effective March 1, 2023; and the NMPA's 2023 announcement on strengthening supervision of entrusted production under the MAH system.

stated that drug-injury victims could seek compensation from the holder, the entrusted manufacturer, or the seller, with recourse among them according to legal responsibility. It also required cross-jurisdiction regulatory coordination when the holder and the entrusted manufacturer were located in different provinces. In institutional terms, the pilot did not simply permit outsourcing; it established the early legal template for authorization-retaining external commercialization.[5]

Second stage: legal consolidation in the 2019 Drug Administration Law. The 2019 revision of the Drug Administration Law converted MAH from a pilot arrangement into a formal legal architecture. The law defines the marketing authorization holder as the entity that obtains the drug registration certificate. It further states that the holder is responsible for the drug's non-clinical research, clinical trials, production and distribution, post-marketing studies, and pharmacovigilance-related monitoring, reporting, and handling. The law also states that the holder's legal representative and principal person are fully responsible for drug quality.

The same chapter then adds three institutional elements that are central for this paper. First, the holder must establish a drug quality assurance system and assign dedicated personnel to quality management. Second, the holder must regularly audit the quality-management systems of entrusted manufacturers and distributors and supervise them to ensure that they continue to possess quality assurance and control capabilities. Third, the law explicitly allows the holder either to manufacture by itself or to entrust manufacturing to a qualified drug manufacturer, but in the entrusted case the holder and the manufacturer must sign both an entrustment agreement and a quality agreement and strictly perform the obligations set out in those agreements.[6]

Third stage: post-law supervisory deepening. Subsequent NMPA rules sharpened the operational meaning of holder responsibility. The 2022 supervisory provisions on the holder's primary responsibility for drug quality and safety emphasize that the holder must establish a full-process quality-management system and remain responsible for the drug's

[5]See the State Council's June 6, 2016 announcement on the pilot plan. The official release states that R&D institutions or scientific personnel in pilot regions could apply for authorization and become holders; that victims could seek compensation from the holder, entrusted manufacturer, or seller; and that the holder's province and the manufacturer's province should coordinate supervision.

[6]The official law text, as published after the 2019 revision, states in the MAH chapter that the holder is the entity obtaining the registration certificate; that the holder bears responsibility for non-clinical research, clinical trials, production and distribution, post-marketing research, and adverse-reaction monitoring and handling; that the legal representative and principal person bear full responsibility for drug quality; that the holder shall establish a quality assurance system and assign independent quality-management personnel; that the holder shall regularly audit entrusted manufacturers and distributors; and that entrusted production requires both an entrustment agreement and a quality agreement.

safety, effectiveness, and quality controllability across research, production, distribution, use, post-marketing study, and pharmacovigilance. The official policy interpretation also stresses that the holder should maintain an independent quality-management department and clearly assign responsibilities across key functions. In 2023, the NMPA issued a further notice specifically aimed at strengthening supervision of entrusted production under the MAH system. This is important for economic interpretation: the delegated-production route was not converted into a casual or light-touch outsourcing margin. It was converted into a more standardized, more explicitly governed, and more directly supervised route.[7]

A.2 Holder responsibility: what remains with the holder after production is separated

For the economics of the paper, the most important institutional point is that MAH separates *production performance* from *ultimate holder responsibility*; it does not transfer the core accountability for the drug away from the holder.

The legal record implies that, once a drug is authorized, the holder remains the focal entity for the drug across the life cycle. At a minimum, four dimensions remain holder-side responsibilities.

1. **Lifecycle responsibility.** The holder remains the legally central entity from research and registration through post-marketing monitoring.

2. **Quality-system responsibility.** The holder must establish and maintain a quality assurance system rather than rely passively on the contractor's internal systems.

3. **Oversight responsibility.** The holder must audit and supervise the quality-management systems of entrusted manufacturers and other entrusted parties.

4. **Release and safety responsibility.** The holder remains the entity on which the law and the supervisory rules center quality and safety responsibility, even when physical production is performed elsewhere.

This institutional structure is precisely why the external route is economically subtle. MAH did not create a world in which a research-originating firm can simply hand off production and walk away. The holder retains legally meaningful obligations. In fact, one

[7]The NMPA's 2022 announcement releasing the supervisory provisions states that the rules would take effect on March 1, 2023. Official policy interpretations emphasize full-process quality management, an independent quality-management department, and clearer responsibility for key posts. The NMPA's 2023 announcement on entrusted production further tightened supervision of MAH-based delegated manufacturing.

reason the reform is economically interesting is that it makes the external route feasible *without* eliminating holder-side responsibility. That is also why the relevant model object is not a zero-cost outsourcing technology. The external route remains costly, but the cost is now attached to a standardized and legally recognized governance structure rather than to an ad hoc or weakly enforceable arrangement.

A.3 Entrusted production: why it is more than a make-or-buy choice

The term "entrusted production" can sound deceptively simple, as if the reform merely allowed a firm to buy manufacturing services in the market. Institutionally, the arrangement is more specific. The MAH architecture requires that entrusted production be carried out by a qualified drug manufacturer and that the relationship between holder and manufacturer be governed by both an entrustment agreement and a quality agreement. In other words, external commercialization is not just a spot purchase of factory capacity. It is a legally structured relationship in which quality responsibility, monitoring obligations, and execution requirements must be allocated and performed.

This point helps explain why the external route should be modeled through a governance and compliance object rather than through a single production-price term. Even if the market payment to a qualified manufacturer is represented by p_m, the holder must still incur additional costs of selecting, auditing, coordinating, documenting, and supervising the cross-firm arrangement. These costs are distinct from the physical act of producing a batch of drugs. The official law and the later supervisory rules make exactly this distinction visible: the contractual and quality architecture of the relationship is an object of regulation in its own right.

The institutional logic can therefore be written schematically as follows:

MAH = separation of authorization and productionunder retained holder responsibility and standardized

This is the core legal-to-economic bridge of the paper.

A.4 Why the policy primitive is route-specific

Once the institutional content is stated precisely, the natural policy primitive follows. Let a research-originated original-drug idea be owned by a research-oriented firm. There are two conceptually distinct ways to complete commercialization.

1. **Internal implementation.** The project is carried out inside an integrated structure.

2. **External commercialization.** The innovator retains the relevant holder role and completes commercialization through a qualified outside manufacturer.

The institutional content of MAH concerns the second route. Therefore the natural primitive is not "the reform" in the abstract, but the cost attached to that route. Denote this cost by

$$\tau_{ext} \geq 0.$$

By construction, τ_{ext} is a route-level object. It is not total firm-level governance cost, not aggregate regulatory burden, and not an omnibus policy wedge.

The logic is easiest to see by writing the external route separately from the internal route. Let $R_B(z)$ denote the private value of successful commercialization for a research-oriented firm with capability state z. Let p_m denote the price of qualified contract manufacturing. Let $\lambda_{ext}(z, p_m) \in [0, 1]$ denote the route-level success rate of external commercialization, where success here refers to a reduced-form implementation or realization probability conditional on using that route. Then the expected net value of one idea under external commercialization can be written as

$$H_B^{ext}(z, p_m; \tau_{ext}) = \lambda_{ext}(z, p_m) R_B(z) - p_m - \tau_{ext}. \tag{1}$$

This decomposition is useful because each term has a distinct interpretation:

- $R_B(z)$: value of a successful original-drug commercialization outcome,
- $\lambda_{ext}(z, p_m)$: technical or execution-side realization term,
- p_m: market payment to the outside manufacturer,
- τ_{ext}: governance and compliance burden of using the external route.

Equation (1) is exactly where the institutional interpretation of MAH belongs. The reform lowers τ_{ext}, not necessarily p_m, and not necessarily the idea-generation cost of science itself.

A.5 Why τ_{ext} is not demand, science, or a generic fixed cost

The institutional record also clarifies what the reform is *not*.

Not a demand shock. The legal core of MAH concerns who may hold authorization, who may manufacture, how entrusted production is governed, and who bears quality responsibility. These are organizational and regulatory-allocation changes. They do not, by

themselves, change reimbursement rules, final demand conditions, or therapeutic demand for a drug. A demand channel could of course coexist in data through other policies, but it is not the distinctive institutional content of MAH.

Not a scientific-productivity shock. Nothing in the legal architecture of MAH changes the research technology by which an idea is scientifically discovered. The reform may raise the expected payoff to research because more ideas become commercializable, but that is an expected-return channel, not a direct productivity shift in the scientific production function.

Not a generic fixed-cost reduction for all firms. This point is especially important. MAH does not simply relax every firm's operating burden. The post-law architecture may even impose more explicit documentation, auditing, and quality-system obligations on the holder. What changes is that these obligations are attached to a legally recognized and standardized external route, so that the *net friction of using that route* falls relative to the pre-reform environment. In other words, MAH can simultaneously preserve or even sharpen holder-side responsibility while still lowering the effective governance cost of external commercialization by making the arrangement legally supported, contractually structured, and regulatorily intelligible. That is why the correct object is a route-specific friction, not a generic fixed cost common to all firms.

A.6 Per-idea value and route choice

Suppose the research-oriented firm compares external commercialization with at least one outside option. The simplest outside option is to abandon the idea, normalized to zero. A richer version adds internal implementation or organizational switching. To keep the logic transparent, write the per-idea value as

$$\Psi_B(z, p_m; \tau_{ext}) = \max\left\{H_B^{ext}(z, p_m; \tau_{ext}),\ H_B^{int}(z),\ 0\right\}, \tag{2}$$

where $H_B^{int}(z)$ denotes the net value of the best internally organized route available to the firm.

Two useful cases follow.

Case 1: external route is the relevant margin. If the reform mainly operates by opening or lowering the cost of external commercialization, then the relevant comparative static is obtained from the external term in (2). Since

$$\frac{\partial H_B^{ext}}{\partial \tau_{ext}} = -1,$$

a fall in τ_{ext} weakly raises Ψ_B, and strictly raises it whenever the external route is the maximizing route in (2).

Case 2: internal implementation remains an option. If the firm can instead switch into a more integrated structure, then MAH may still matter even without changing the internal route directly. A fall in τ_{ext} lowers the relative value of full internalization and therefore changes organizational assignment even if internal implementation itself is technologically unchanged.

Either way, the point remains the same: the reform operates first on the route-level value of external commercialization, and organizational outcomes move because relative route values move.

A.7 What the data can and cannot identify

The identification boundary in the main text can be restated formally. Let observed outcomes be collected in a vector

$$Y = (N_O, S_O, E_B, T, C_m, Y_C),$$

where N_O is the number of original-drug realizations or closely related counts, S_O is the share of original-drug outcomes, E_B is research-oriented entry, T denotes organizational transition objects, C_m denotes commercialization-route proxies, and Y_C denotes producer-side outcomes for qualified manufacturers.

These objects can move coherently after reform if the reform lowers τ_{ext}. But the mapping from Y to τ_{ext} is not one-to-one without additional structure. The role of the model is therefore to provide a disciplined interpretation of jointly moving margins rather than to claim that a primitive route-specific friction is directly observed in the raw data.

B Full Model Objects and Additional Derivations

Purpose. This appendix corresponds primarily to Section 3. It should contain the detailed classification of states, dynamic controls, static inputs, prices, and payoff objects, as well as the algebra that is too detailed for the main text.

What belongs here.

- Detailed object table.
- Additional derivations for $\bar{\pi}_A$, $\bar{\pi}_C$, Π_A, Π_B, and Π_C.
- Expanded Bellman equations and admissible choice sets.

- Any later extension that remains consistent with the current baseline architecture.

C Proofs and Comparative-Static Details

Purpose. This appendix corresponds to Section 4. It should contain proofs, derivation steps, and any regularity conditions required for the comparative-static propositions.

What belongs here.

- Proof details for the effect of τ_{ext} on route choice.
- Proof details for the effect on type-B research intensity.
- Proof details for innovative entry and producer-side equilibrium responses.

D Empirical Classification, Mapping, and Estimation Details

Purpose. This appendix corresponds to Section 5. It should contain variable construction, classification rules, moment selection, numerical implementation details, and later robustness material.

What belongs here.

- Detailed $A/B/C$ classification rules.
- Transition-matrix construction and validation details.
- Measurement of original-drug outcomes and commercialization proxies.
- Estimation-algorithm details and any later robustness checks.